NUR258SCE Post-Operative Complications Simulation Scenario

(Revised 12/11/2025 CA)

Client: DeShawn Johnson

Discipline: Medical Surgical Nursing II	**Student Level:** Advanced Med-Surg
Expected Simulation Run Time: 5 Hours	**Location for Reflection:** Joyce Briefing Center
Location: Joyce Simulation Center	**File Name:** NUR258SCE Postop Complications. NLN

Brief Description of Patient

Hospital Location: Medical/Surgical-Telemetry **Shift:** Day shift starting at 0700 am.

Name: DeShawn Johnson

Date of Birth: 1/18/19XX	**Age:** 61	**Allergies:** Sulfa: rash
Admit Weight: 105 kg	**Height**: 182 cm	**Pronouns:** He/Him
Marital Status: Divorced	**Language**: English	**Sex:** Male

Attending Provider/Team: Dr. Naomi Patrick

Primary Admitting Medical Diagnosis: Cholecystitis resulting in an open cholecystectomy 3 days prior.

Past Medical History: Current 1 pack a day smoker, Hypertension, Hyperlipidemia, Diabetes Type 2, and Osteoarthritis

History of Present Illness: The client is a 61-year-old male who presented to the emergency department three days ago with complaints of abdominal pain and jaundice. He underwent an open cholecystectomy for obstructive jaundice. Two days ago, the client developed signs of a surgical site infection, and IV vancomycin was initiated. Today is postoperative day three. During the night shift, the client was awake throughout the night and exhibited confusion. His urine output over the past 12 hours was 250 mL.

Social History: Client is divorced and has 2 adult children. He works full-time.

Simulation Learning Objectives

NUR258 Course Outcomes

1. Use critical thinking, clinical reasoning, and clinical judgement to prioritize comprehensive, individualized, and holistic care of the adult client. (CO1)
2. Select appropriate priorities for nursing assessment and interventions when delivering safe nursing care for adults. (CO2)
3. Deliver evidence-based holistic nursing care for adult clients with health alterations. (CO3)
4. Evaluate outcomes of nursing care of adult clients with health alterations. (CO4)

Simulation Scenario Objectives

1. Use critical thinking to identify post-operative complication. (CO1)
2. Based on recognized cues, perform two appropriate interventions for the recognized cues. (CO2)
3. After taking action and performing interventions, reassess the client to ascertain if the performed interventions were successful. (CO4)

Setting/Environment

- Medical-Surgical Unit/Telemetry

Equipment/Supplies for Post-Op Complications: DeShawn Johnson

Simulated Patient/Manikin(s) Needed: HAL

Recommended Mode for Simulator: Pre-programmed, manually advance the scenario.

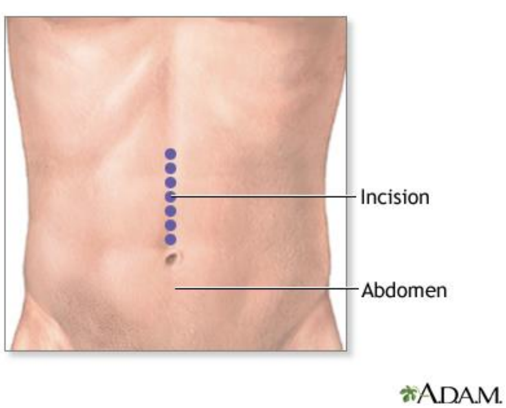
Other Props & Moulage:

Equipment Attached to Manikin/Simulated Patient:

- ID band for DeShawn Johnson.
- Male genitalia
- IV in place and working.
- IV tubing with primary line Vancomycin set up.
- Alaris IV pump with two channels.
- Alaris IV pump channel at 250 mL/hr (1 gram Vancomycin in 250 mL bag of 0.9% NS)
  - Vancomycin should be connected at the lower Y-site running together with bag of NS.
- Alaris IV pump channel at 50mL/hr (500mL bag of 0.9% NS)
- Foley catheter (with a port) in place, 30 ml dark amber urine in bag. Foley secured with STAT lock.

Moulage and manikin programming:

- Yellow/green vomit in emesis basin or emesis bag, about 100 ml.
- 2+ edema, bilateral lower legs
- Red, swollen, midline (in the middle of stomach), vertical incision. Begin 2-3 inches below the ribs and end at belly button (about 6 inches long). **No dressing**.



Equipment Available in Room:

- Equipment to take vitals: Pulse oximeter, telemetry cables (3 lead), thermometer.
- 02 delivery device: nasal cannula and simple mask.
- Toothette or toothbrush
- Blood collection supplies: vacutainer, gauze, tape, flushes, butterfly syringe, & lab tubes.
- Bedpan/urinal
- Black bin to deposit used supplies.
- Emesis basin
- Water cup and drinking straw.
- Suction tubing and suction canister.
- Incentive spirometer
- Grip socks

Additional Equipment Available for Scenario:

- Sequential Compression Device (SCD).

Scenario Programming

- Add PVCs to monitor, 30 per minute (uni).

Vitals

HR: 95	**BP**: 123/72	**Temp**: 36.9
RR: 20	**SpO2**: 89% Room Air	

All Medications and fluids needed for scenario (and quantity of medication)	
Oral Meds • Enalapril Maleate 20 mg, Daily, Oral (2 tabs/patient) Transdermal Patch • Nicotine 21 mg, transdermal patch, change every 24 hours, rotate sites. (1 patch) IV medication • Vancomycin 1 gram/ 200 ml in 0.9% NACL, every 12 hours, IV drip (1 bag)	IV Push • Furosemide 40 mg, IV Push, once, now. (1 vial) • Ondansetron 4 mg, every 4 hours as needed, IV push. (2 vials/patient) • 50% Dextrose in water, 25 grams, now, IV push. (1 syringe) • 10% Calcium Gluconate, 4.5 mEq/ml, once, now, IV Push (1 dose) • Regular Insulin 10 units, 100 units/ml, IV push, now. (1 dose) Green: Need in SimServe Yellow: In the room, running

Roles

Students	**Simulation Staff**
Lead Nurse	Nursing Instructor & SOS/Tech Team acting as following: • Physician • Charge Nurse • Pharmacy • Radiology • Cardiac/EKG Tech • Phlebotomy/Lab • Operator • Patient • Family Members

Guidelines/Information Related to Roles

All Nursing Students:

- Employee the TeamSTEPPS principles of leadership, team structure, communication, situation monitoring, and mutual respect.
- Monitor the client's condition and perform interventions as appropriate.
- Determine which assessments and interventions must be performed by the lead nurse.
- Determine and assign/delegate appropriate tasks according to the scenario.
- Set appropriate goals and objectives for scenario.
- Complete a client head to toe assessment and interpret findings of assessment, diagnostics, and lab values.
- Practice closed loop communication and SBARR.

Pre-briefing/Briefing

Students and instructor will discuss the following in pre-briefing:

1. Care of the post-operative client.
2. Priorities based on client cues.
3. Possible organ dysfunction and cues.
4. Physical assessment and client cues.
5. Expected and concerning lab results.
6. Expected and concerning diagnostic results.
7. Forming a hypothesis and making recommendations based on client cues.
8. Expected medications for client.

Student Shift Report: **verbally** given to students by instructor.

Client Name	**Code Status**	**Allergies**	**DOB**	**Rm #**	**MD**
DeShawn Johnson	FULL	Sulfa: rash	1/18/XX **Age** 61	TBD	Naomi Patrick

Vital Signs (last set done at 0600 am) **HR** 98 **O2** 91% Room Air **BP** 108/72 **Temp** 36.5 C **RR** 20	**Home Medications** • Enalapril • Atorvastatin • Aspirin • Metformin **Health History** • Hypertension (high blood pressure) • Hyperlipidemia (elevated cholesterol) • Osteoarthritis (Joint Arthritis) • Diabetes Type 2

Admitting Diagnosis: Open Cholecystectomy 3 days ago

The client is a 61-year-old male who presented to the emergency department three days ago with complaints of abdominal pain and jaundice. He underwent an open cholecystectomy for obstructive jaundice. Two days ago, the client developed signs of a surgical site infection, and IV vancomycin was initiated. Today is postoperative day three. The client was awake throughout the night and exhibited confusion. An attempt was made to contact Dr. Patrick to update her on the client's status and laboratory results, but she did not answer. Please return the call and provide an update.

Neuro Awake and confused. A&O x 2 (person and place) He wasn't confused like this when night shift started. It got worse overnight. Pupils are PERRLA.	**Cardiology** Sinus tachycardia, S1&S2 Occasional "weird blips" on tele. BP stable.
Respiratory Breath sounds are clear, equal, and rapid. Occasional crackles, encourage IS.	**GI** Nauseated and vomited x2 (total of 100 ml) Last BM yesterday during day shift.
Skin/Wounds Warm, dry, and pale Incision: red, swollen, and warm with purulent discharge. Open to air.	**GU** Foley in place. Total output is 250 ml on night shift. Anything in the bag will count for your shift.
Drains/IVs 20 gauge Left Forearm, intact. Vancomycin running. It was started at 0600 and should be done at 0800.	**Diagnostics** Lab set #1 was received at 0500 am. Unable to reach MD during night shift. Left a voice mail.
Pain 3/10 Unable to explain if it is his stomach or incision because of confusion.	**Fall Risk** High Fall Risk. Only get out of bed with physical therapy.
Psycho/Social Divorced and 2 adult children. Smokes 1 pack of cigarettes per day. Frequently asks for a cigarette.	**Diet** Regular diet

ATI: INITIAL ORDERS (Everything listed below will already be visible to the student in ATI when the scenario is started.)

Medications Visible Pre-Scenario	**Medications** • Enalapril Maleate 20 mg, Daily, Oral • Ondansetron 4 mg, every 4 hours as needed, IV push. • Vancomycin 1 gram/ 250 ml in 0.9% NACL, every 12 hours, IV drip. • Nicotine 21 mg, transdermal patch, change every 24 hours, rotate sites. (1 patch) • Metformin 500 mg tablet, BID, PO
Orders visible pre-scenario	**Nursing/Misc. orders** • Vital signs: every 2 hrs. • Continuous cardiac monitoring (telemetry). • Respiratory: supplemental oxygen therapy to keep SPO2 greater than or equal to 92% • Incentive Spirometry, hourly while awake. • Sequential Compression Device (SCD): SCD on client when client is in bed.
Labs visible pre-scenario	**Post-Op Day 1 Labs** **CBC** — **Result** — **Normal** WBC 12 5-10 Hgb 17 14-18 Hct 51% 42-52 Platelets 250 150-400 **Chemistry/ Metabolic** Na 141 135-145 K 4.0 3.5-5 Cl 99 98-106 CO2 24 23-29 BUN 16 10-20 Creatinine 1.1 0.6-1.2 Glucose 96 60-100 Ca 9.5 9-10.5 PO 4.0 3.5-4.5 Mg 2.1 1.3-2.3
	Post-Op Day 2 Labs **CBC** — **Result** — **Normal** WBC 21 5-10 Hgb 15 14-18 Hct 45% 42-52 Platelets 258 150-400 **Chemistry/ Metabolic** Na 140 135-145 K 5.0 3.5-5 Cl 99 98-106 CO2 27 23-29 BUN 25 10-20

	Creatinine 2.1 0.6-1.2 Glucose 82 60-100 Ca 9.8 9-10.5 PO 4.1 3.5-4.5 Mg 2.0 1.3-2.3
	Post-Op Day 3 Labs: Day of Scenario **CBC** **Result** **Normal** WBC 15 5-10 Hgb 15 14-18 Hct 45% 42-52 Platelets 258 150-400 **Chemistry/ Metabolic** Na 141 135-145 K 6.7 3.5-5 Cl 99 98-106 CO2 18 23-29 BUN 50 10-20 Creatinine 3.25 0.6-1.2 Glucose 102 60-100 Ca 10.1 9-10.5 PO 4.4 3.5-4.5 Mg 2.3 1.3-2.3

ATI: AVAILABLE SCENARIO ORDERS (these will all be available to push out in the scenario)

Medications available to order during scenario.	• Furosemide 40 mg IVP, once, now. • 50% Dextrose in water, 25 grams, now, IV push. • Calcium Gluconate 10%, 1 gram/10mL, once, now, IV Push • Regular Insulin 10 units, 100 units/ml, IV push, now.
Orders available to order during scenario.	• 12 Lead EKG, STAT • Discontinue Vancomycin • CMP 2 hours after administering furosemide.
Lab results available to push out during scenario.	None

Scenario Progression Outline

Patient Name: DeShawn Johnson **Date of Birth:** 1/18/19XX
Doctor Name: Naomi Patrick, MD **Allergies:** Sulfa: rash

*****Instructor needs to start the Vancomycin PRIOR to starting the scenario.*****

Phase	Manikin/SP Actions	Expected Nursing Interventions	MD Orders and Student Cues
Phase 1	**Vitals** **HR:** 118 **BP:** 102/65 **RR:** 20 **SpO2:** 89% RA **Temp:** 36.9 C **Pain:** 2/10 at incision • Add PVCs to the client's rhythm on the monitor because of hyperkalemia. Program manikin with PVC (uni) 30/min. ***NOTE: all systems are WNL, except what it listed below. • **Neuro**: PERRLA, follows commands, confused (situation, time, & place). • **Cardio:** PVC's • **Abdomen**: Incision is red, swollen, purulent drainage, & open to air. • **GI/GU**: 30 ml dark, amber urine. • **Emesis**: Yellow/green emesis (vomit) in an emesis basin/bag. • 2+ edema, bilateral lower legs	**Prior to calling the MD the students should do the following:** ❑ Review lab values. ❑ Obtain vital signs (BP, HR, O2, Temp, RR). ❑ Perform assessment. ❑ Hold Enalapril. ❑ Administer Ondansetron. ❑ Form a hypothesis and plan of an SBAR with recommendations **prior** to calling MD. **Scenario objectives:** Use critical thinking to identify post-operative complications. (CO 1) ❑ Elevated creatine ❑ Elevated BUN ❑ Elevated potassium ❑ Low urine output ❑ Abnormal EKG Based on recognized cues, perform two appropriate interventions for recognized cues. (CO 2) ❑ Calcium gluconate ❑ Insulin ❑ Dextrose ❑ Furosemide. After taking action and performing interventions, reassess the client to ascertain if the performed interventions were successful (even if there is no change, did the students reassess?): ❑ Increased urine output ❑ Improved oxygenation ❑ Improved lung sounds ❑ Improved mentation.	**Student Cues** • The client is nauseated & actively vomiting. • The client is anxious and confused (He thinks it's Christmas this weekend and keeps asking about Christmas. • Client only knows his name, DOB & and that he had surgery of some kind. **MD Orders** • Stat 12 lead EKG. • Discontinue all vancomycin. • CMP 2 hours after administering furosemide • Calcium Gluconate 10%, 1gram/10mL, now IV push. • 50% Dextrose in water, 25 grams, now, IV push. • Regular insulin, 10 units, IV push, IV push, now. • Furosemide 40 mg IV push, now. • Make client NPO (nothing by mouth).

Debriefing/Guided Reflection

Debriefing Phase	Debriefing Questions for Consideration
Reactions/ Defuse	How did you feel throughout the simulation experience?
	Give a brief summary of this patient and what happened in the simulation.
	What were the main problems that you identified?
Analysis/ Discovery	Discuss the knowledge guiding your thinking surrounding these main problems.
	What were the key assessments and interventions for this patient?
	Discuss how you identified these key assessments and interventions.
	Discuss the information resources you used to assess this patient. How did this guide your care planning?
	Discuss the clinical manifestations evidenced during your assessment. How would you explain these manifestations?
	Explain the nursing management considerations for this patient. Discuss the knowledge guiding your thinking.
	What information and information management tools did you use to monitor this patient's outcomes? Explain your thinking.
	How did you communicate with the patient?
	What specific issues would you want to take into consideration to provide for this patient's unique care needs?
	Discuss the safety issues you considered when implementing care for this patient.
	What measures did you implement to ensure safe patient care?
	What other members of the care team should you consider important to achieving good care outcomes?
	How would you assess the quality of care provided?
	What could you do to improve the quality of care for this patient?
Summary/ Application	If you were able to do this again, how would you handle the situation differently?
	What did you learn from this experience?
	How will you apply what you learned today to your clinical practice?
	Is there anything else you would like to discuss?

References

Hinkle, J. and Cheever, K. (2018). Brunner & Suddarth's textbook of medical-surgical nursing (14th ed.). Philadelphia, PA: Lippincott (pp.818-832)

Assessment Technologies Institute (2019). RN adult medical-surgical nursing (11th ed.). Still, KS: Author. Available from https://www.atitesting.com (pp.205-212)

Section	Content
Scenario Name	Post-Operative Complications – Acute Kidney Injury with Hyperkalemia (DeShawn Johnson)
Setting	Medical-Surgical Telemetry Unit post-op open cholecystectomy patient
Primary Learning Focus	Recognition of post-operative deterioration with emphasis on acute kidney injury hyperkalemia infection-related complications prioritization and reassessment
Priority Hypothesis	Acute kidney injury with severe hyperkalemia likely worsened by vancomycin nephrotoxicity and low urine output in the setting of post-operative infection
Secondary Hypotheses	Post-operative infection dehydration poor perfusion early fluid overload or worsening respiratory compromise
Key Supporting Cues	Confusion low urine output elevated creatinine elevated BUN potassium 6.7 PVCs hypoxia nausea vomiting purulent incision dark amber urine bilateral edema
Red Flags / Escalation Triggers	Potassium 6.7 PVCs worsening confusion urine output 250 mL over night or 30 mL in bag SpO2 below 92 percent creatinine 3.25 BUN 50 active vomiting and hypoxia
Expected Clinical Progression if Missed	Acute kidney injury worsening hyperkalemia dysrhythmias decreased perfusion respiratory compromise cardiac arrest and death
Common Student Errors	Missing the significance of urine output focusing only on the wound infection delaying recognition of hyperkalemia not connecting PVCs to potassium not holding enalapril not reassessing after treatment
Clinical Judgment Focus	Linking lab abnormalities to cardiac and renal findings recognizing hyperkalemia as the immediate threat prioritizing stabilization and reassessing after interventions
Real-World Translation	This patient would likely trigger urgent provider notification telemetry monitoring EKG review discontinuation of nephrotoxic medication treatment of hyperkalemia and close monitoring for renal and cardiac deterioration
Instructor Teaching Emphasis	Hyperkalemia is the immediate danger low urine output and rising creatinine signal kidney injury students must treat the lethal problem first and reassess rhythm mentation oxygenation and urine output

Post-Operative Complications DeShawn Johnson

Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	SBAR	Teaching/Debrief Focus	High Risk	Common Missed Items
Scenario Overview	Open cholecystectomy 3 days ago	Post-operative patient at risk for infection fluid shifts and organ dysfunction	Important	Provides context for current deterioration	Missed post-op complication recognition	Monitor for complications and trends	Ongoing	B	Post-op day 3 should prompt concern for complications		No
Scenario Overview	Surgical site infection on vancomycin	Known infection source with possible medication-related renal impact	Important	Important contributor to current clinical picture	Sepsis progression or nephrotoxicity	Assess wound infection and medication effect	Ongoing	B	Infection and treatment can both create risk	✓	Yes
Scenario Overview	Primary scenario focus = hyperkalemia with AKI	Core teaching point of scenario	Immediate	Life-threatening electrolyte abnormality with renal failure and dysrhythmia risk	Cardiac arrest and worsening organ dysfunction	Recognize emergent pattern and intervene quickly	Ongoing	A R	This is not just a wound infection scenario it is a hyperkalemia and AKI emergency	✓	Yes
Baseline / Shift Report	Full code	Background for escalation planning	Relevant	Important background only	Minimal	Know code status	N/A	S	Background awareness only		No
Baseline / Shift Report	Sulfa rash allergy	Medication safety background	Relevant	Important for safe prescribing but not current priority	Medication error	Verify allergy before treatments	N/A	B	Always verify allergies		No
Baseline / Shift Report	Age 61	Older adult with post-op risk and chronic comorbidities	Relevant	General context only	Minimal	Routine age-appropriate care	N/A	B	Context only		No
Baseline / Shift Report	MD Naomi Patrick	Provider for escalation and orders	Relevant	Needed for communication and SBAR	Delay in escalation	Know who to call	N/A	R	Communication readiness		No
Baseline / Shift Report	HR 98 baseline	Tachycardia trend begins before phase 1	Important	Useful comparison point	Miss worsening instability	Trend HR	Ongoing	A	Trends matter		No
Baseline / Shift Report	O2 91 percent RA baseline	Hypoxia already present at report	Immediate	Respiratory compromise already exists	Delayed oxygen support and worsening hypoxia	Apply oxygen and trend saturation	Frequent reassessment	A	Low oxygen at baseline should not be ignored	✓	Yes
Baseline / Shift Report	BP 108/72 baseline	Perfusing but not robust	Important	Useful for comparison with later drop	Miss hemodynamic decline	Trend BP	Ongoing	A	Trend awareness		No
Baseline / Shift Report	RR 20 baseline	Normal rate but must be trended with oxygenation	Relevant	Useful comparison point	Miss respiratory worsening	Trend RR	Ongoing	A	Single normal value can be misleading		No

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Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	SBAR	Teaching/Debrief Focus	High Risk	Common Missed Items
Baseline / Shift Report	Enalapril home med	ACE inhibitor can worsen potassium and affect renal status	Important	Should be held with hyperkalemia and AKI	Worsening hyperkalemia and renal injury	Hold medication and notify provider	Reassess labs and BP	B R	Medication safety with abnormal labs matters	✓	Yes
Baseline / Shift Report	Atorvastatin home med	Chronic medication background	Relevant	Not driving acute scenario	Minimal	Routine	N/A	B	Low priority medication		No
Baseline / Shift Report	Aspirin home med	Chronic medication background	Relevant	May matter for bleeding risk but not current priority	Minimal	Review only	N/A	B	Low priority medication		No
Baseline / Shift Report	Metformin home med	Diabetes medication	Important	May need holding in acute illness or renal dysfunction	Medication safety issue	Review renal status and NPO status	Reassess plan	B	Renal dysfunction changes med safety		Yes
Baseline / Shift Report	Hypertension history	Chronic cardiovascular disease	Relevant	Important when deciding to hold Enalapril	Potential perfusion issues	Monitor BP	Ongoing	B	History influences decisions		No
Baseline / Shift Report	Hyperlipidemia history	Chronic condition	Relevant	Not current priority	Minimal	Routine	N/A	B	Low priority history		No
Baseline / Shift Report	Osteoarthritis history	Chronic condition	Relevant	Not current priority	Minimal	Routine	N/A	B	Low priority history		No
Baseline / Shift Report	Diabetes type 2 history	Comorbidity affecting healing and infection risk	Important	Contributes to post-op complication risk	Delayed healing and worsening infection	Monitor glucose and healing	Ongoing	B	Diabetes worsens outcomes		No
Baseline / Shift Report	Confusion worsening overnight	Acute neuro change and possible uremia hypoxia or electrolyte effect	Immediate	New confusion indicates significant deterioration	Missed encephalopathy or worsening instability	Perform neuro assessment and investigate cause	Frequent reassessment	A	Acute confusion is a red flag not just delirium	✓	Yes
Baseline / Shift Report	A&O x2 baseline at report	Abnormal mental status baseline for scenario	Immediate	Needed to trend mentation changes	Miss worsening or improvement	Trend LOC	Ongoing	A	Mentation is an outcome measure	✓	Yes
Baseline / Shift Report	Sinus tachycardia	Stress response and possible reduced perfusion	Important	Supports overall instability	Miss worsening status	Trend cardiac status	Ongoing	A	Physiology matters even before PVCs		No
Baseline / Shift Report	Occasional weird blips on tele	Early cue for dysrhythmia likely related to hyperkalemia	Immediate	May represent PVCs and cardiac instability	Progression to lethal rhythm	Assess telemetry obtain EKG promptly	Reassess rhythm	A R	Students often under-react to vague telemetry clues	✓	Yes

Post-Operative Complications DeShawn Johnson

Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	SBAR	Teaching/Debrief Focus	High Risk	Common Missed Items
Baseline / Shift Report	BP stable at report	May falsely reassure students	Relevant	Must be viewed with other abnormal findings	False reassurance and delay	Continue full assessment	Ongoing	B	Stable BP does not mean stable patient		Yes
Baseline / Shift Report	Breath sounds clear equal and rapid	Respiratory assessment shows tachypnea without focal findings	Important	Supports systemic problem not isolated lung process	Miss broader cause	Assess oxygenation and work of breathing	Reassess after oxygen	B	Respiratory findings must be interpreted in context		No
Baseline / Shift Report	Occasional crackles	Possible fluid overload or evolving pulmonary congestion	Important	Important with edema and renal failure	Miss worsening fluid status	Assess lungs and fluid balance	Reassess after furosemide	B	Crackles plus edema plus low urine output suggest volume issue	✓	Yes
Baseline / Shift Report	Nausea and vomiting x2 100 mL	Common hyperkalemia and uremia symptom also risk for aspiration	Important	Signals patient distress and need for symptom treatment	Aspiration ongoing losses or delayed meds	Administer ondansetron and protect airway	Reassess nausea	B	Do not ignore symptom burden in unstable patient		No
Baseline / Shift Report	Last BM yesterday	Baseline GI history	Relevant	Not current priority	Minimal	Routine	N/A	B	Low priority finding		No
Baseline / Shift Report	Warm dry pale skin	Abnormal pallor with systemic illness	Important	Supports poor overall status	Miss severity of illness	Assess perfusion and oxygenation	Ongoing	B	Pallor is another global cue		No
Baseline / Shift Report	Incision red swollen warm purulent open to air	Active wound infection	Important	Infection source remains important but not the only problem	Sepsis progression	Assess wound and report findings	Ongoing	B	Do not ignore source control while treating hyperkalemia	✓	Yes
Baseline / Shift Report	Foley in place	Allows accurate urine output monitoring	Important	Urine output is a key marker of renal perfusion	Miss oliguria	Maintain and monitor Foley	Ongoing	B	Foley purpose matters in AKI		No
Baseline / Shift Report	Urine output 250 mL overnight	Oliguria and renal dysfunction clue	Immediate	Low urine output plus rising creatinine indicates AKI	Worsening renal injury and delayed treatment	Trend I&O and notify provider	Reassess urine output	A R	Low urine output is a major clue	✓	Yes
Baseline / Shift Report	20 gauge left forearm IV intact	Access for urgent medications available	Important	Needed for hyperkalemia treatment	Delayed rescue meds	Verify IV patency	Reassess site	B	Always verify access early		No
Baseline / Shift Report	Vancomycin running 0600 to 0800	Nephrotoxic medication currently infusing	Immediate	Must be stopped with suspected vancomycin-associated AKI	Continued kidney injury	Stop infusion when ordered and notify provider	Reassess renal plan	A R	Students often miss the need to discontinue the offending agent	✓	Yes

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Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	SBAR	Teaching/Debrief Focus	High Risk	Common Missed Items
Baseline / Shift Report	Lab set #1 received at 0500	Abnormal labs already available before bedside intervention	Immediate	Students are expected to use existing data	Delayed recognition and poor SBAR	Review labs before calling provider	N/A	R	Available data should guide nursing judgment	✓	Yes
Baseline / Shift Report	Pain 3 out of 10 unable to localize due to confusion	Low pain but unreliable history because of confusion	Relevant	Not current priority	Minimal	Assess broadly and do not anchor on pain	N/A	B	Confusion limits symptom reliability		Yes
Baseline / Shift Report	High fall risk only out of bed with PT	Safety concern worsened by confusion	Important	Confusion increases fall risk	Injury or line removal	Implement fall precautions	Reassess safety	B	Neuro changes increase safety needs		No
Baseline / Shift Report	Divorced with 2 adult children	Psychosocial context	Relevant	Background only	Minimal	Include if needed	N/A	B	Holistic care		No
Baseline / Shift Report	Smokes 1 pack per day asks for cigarette	Nicotine dependence and coping issue	Relevant	Background but may distract from urgent priorities	Delay if over-focused	Address after stabilization	N/A	B	Not every request is a priority during instability		Yes
Baseline / Shift Report	Regular diet order	Outdated order based on current instability	Relevant	Will change after provider orders	Potential aspiration or unsafe intake	Anticipate NPO after escalation	Reassess orders	B	Orders change with patient condition		Yes
Pre-Scenario Orders	Enalapril 20 mg PO daily	ACE inhibitor contributes to hyperkalemia risk	Important	Should be held in hyperkalemia and AKI	Worsening potassium and renal injury	Hold medication and communicate reason	Reassess labs and BP	B R	Students should know when to hold meds	✓	Yes
Pre-Scenario Orders	Ondansetron IV PRN	Appropriate treatment for nausea/vomiting	Important	Improves comfort and reduces aspiration risk	Ongoing vomiting and distress	Administer as ordered	Reassess nausea	B	Supportive care still matters		No
Pre-Scenario Orders	Vancomycin IVPB	Treating infection but potentially nephrotoxic here	Important	Medication may worsen renal function	Continued AKI if not stopped	Anticipate discontinuation	Reassess infusion status	B	Medication can become harmful		Yes
Pre-Scenario Orders	Nicotine patch	Comfort measure for nicotine dependence	Relevant	Not urgent in unstable patient	Distraction from priorities	Address later	N/A	B	Avoid prioritizing comfort over instability		Yes
Pre-Scenario Orders	Metformin 500 mg BID	Diabetes medication	Important	May need to be held with NPO or renal dysfunction	Medication safety issue	Review appropriateness with provider	Reassess plan	B	Acute illness changes med safety		Yes

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Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	SBAR	Teaching/Debrief Focus	High Risk	Common Missed Items
Pre-Scenario Orders	Vitals every 2 hours	Routine monitoring order	Important	Too infrequent for unstable patient	Delayed recognition of worsening	Increase frequency based on status	Trend vitals	A	Nursing judgment can exceed routine orders	✓	Yes
Pre-Scenario Orders	Continuous telemetry	Required for dysrhythmia detection	Immediate	Hyperkalemia can cause lethal arrhythmias	Missed rhythm deterioration	Apply and interpret telemetry	Ongoing	A	Telemetry must be interpreted not just attached	✓	Yes
Pre-Scenario Orders	Oxygen to keep SpO2 >= 92 percent	Clear oxygen goal	Immediate	Hypoxia already present	Delayed oxygen support	Apply oxygen promptly	Reassess SpO2	A	Oxygen threshold should trigger action	✓	Yes
Pre-Scenario Orders	Incentive spirometry hourly while awake	Supportive pulmonary order	Relevant	Not primary treatment for instability	Distraction from urgent priorities	Use when stable	Reassess respiratory status	B	Supportive care after priorities		No
Pre-Scenario Orders	SCD while in bed	DVT prevention	Relevant	Standard supportive care	Minimal	Ensure use when appropriate	Ongoing	B	Basic nursing care still matters		No
Pre-Scenario Labs	Post-op day 1 WBC 12	Mildly elevated postop WBC	Relevant	Trend data only	Minimal	Trend over days	N/A	B	Trends matter		No
Pre-Scenario Labs	Post-op day 1 creatinine 1.1	Near baseline renal function	Important	Useful comparison for current AKI	Miss progression	Compare to day of scenario	N/A	B	Renal trends tell the story		No
Pre-Scenario Labs	Post-op day 2 WBC 21	Significant infection trend	Important	Shows worsening inflammatory response	Miss infection progression	Review trend	N/A	B	Infection worsened before current day		No
Pre-Scenario Labs	Post-op day 2 potassium 5.0	Borderline hyperkalemia day before scenario	Important	Early warning sign	Miss progression to severe hyperkalemia	Trend labs	N/A	B	Early abnormal values matter		Yes
Pre-Scenario Labs	Post-op day 2 BUN 25	Renal function worsening	Important	Supports evolving AKI	Miss renal decline	Trend renal labs	N/A	B	AKI developed over time		No
Pre-Scenario Labs	Post-op day 2 creatinine 2.1	Established renal injury by day 2	Immediate	Strong clue before current emergency	Miss progression and cause	Recognize AKI trend	N/A	B	Students should not treat day-of labs in isolation	✓	Yes
Pre-Scenario Labs	Post-op day 3 WBC 15	Still elevated WBC	Important	Supports ongoing infection	Miss ongoing infection source	Monitor and communicate	N/A	B	Source still matters		No
Pre-Scenario Labs	Post-op day 3 potassium 6.7	Severe hyperkalemia	Immediate	Life-threatening dysrhythmia risk	Cardiac arrest	Initiate hyperkalemia treatment urgently	Reassess rhythm and labs	A R	Potassium this high is an emergency	✓	Yes

Post-Operative Complications DeShawn Johnson

Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	SBAR	Teaching/Debrief Focus	High Risk	Common Missed Items
Pre-Scenario Labs	Post-op day 3 CO2 18	Metabolic acidosis component	Important	Supports systemic illness and renal dysfunction	Worsening instability	Include in clinical interpretation	N/A	B	Acid-base status can worsen hyperkalemia		No
Pre-Scenario Labs	Post-op day 3 BUN 50	Severe azotemia	Immediate	Significant renal dysfunction	Miss severity of AKI	Communicate and treat cause	Reassess renal plan	A	AKI severity is obvious in this value	✓	Yes
Pre-Scenario Labs	Post-op day 3 creatinine 3.25	Severe acute kidney injury	Immediate	Major indicator of renal failure	Continued renal damage and poor clearance	Treat contributing factors stop nephrotoxins and monitor output	Reassess renal status	A R	Creatinine trend is central to the scenario	✓	Yes
Pre-Scenario Labs	Post-op day 3 glucose 102	Near normal glucose	Relevant	Not current priority	Minimal	Routine	N/A	B	Not all labs are problems		No
Pre-Scenario Labs	Post-op day 3 calcium 10.1	Normal-high calcium	Relevant	Not driving scenario	Minimal	Routine	N/A	B	Low priority lab		No
Pre-Scenario Labs	Post-op day 3 magnesium 2.3	Normal magnesium	Relevant	Does not explain PVCs	Minimal	Review only	N/A	B	Normal labs help narrow cause		No
Available Orders	Furosemide 40 mg IV push now	Promotes renal potassium excretion and reduces volume overload	Immediate	Part of emergent hyperkalemia treatment	Continued hyperkalemia and fluid overload	Administer after provider order	Reassess urine output lungs potassium response	A R	Furosemide is used to remove potassium through urine	✓	Yes
Available Orders	D50 25 grams IV push	Prevents hypoglycemia when insulin given and supports potassium shift	Immediate	Essential with IV insulin	Hypoglycemia or ineffective treatment	Administer with insulin per order	Reassess glucose and rhythm	A R	Insulin for hyperkalemia is paired with dextrose	✓	Yes
Available Orders	Calcium gluconate 10 percent IV push	Stabilizes cardiac membrane in hyperkalemia	Immediate	First cardiac-protective treatment	Progression to lethal arrhythmia	Administer promptly	Reassess rhythm and telemetry	A R	Calcium protects the heart it does not lower potassium	✓	Yes
Available Orders	Regular insulin 10 units IV push	Shifts potassium intracellularly	Immediate	Core hyperkalemia treatment	Persistent severe hyperkalemia	Administer with dextrose and monitor glucose	Reassess glucose rhythm and labs	A R	Know what each hyperkalemia med does	✓	Yes
Available Orders	Stat 12 lead EKG	Confirms cardiac effect of hyperkalemia	Immediate	Needed with PVCs and K 6.7	Missed dangerous conduction changes	Obtain EKG promptly	Interpret results	A R	Abnormal potassium requires EKG evaluation	✓	Yes
Available Orders	Discontinue Vancomycin	Removes possible nephrotoxic contributor	Immediate	AKI may worsen if continued	Continued renal injury	Stop medication as ordered	Reassess med plan	A R	Stop the harmful contributor	✓	Yes

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Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	SBAR	Teaching/Debrief Focus	High Risk	Common Missed Items
Available Orders	CMP 2 hours after furosemide	Measures treatment response	Important	Needed to evaluate effectiveness	Miss persistent hyperkalemia or worsening renal function	Obtain repeat CMP	Review follow-up results	R	Reassessment includes labs not just bedside signs	✓	Yes
Phase 1 Deterioration	HR 118	Tachycardia	Important	Reflects overall stress and illness	Miss worsening instability	Trend HR	Ongoing	A	Trend matters		No
Phase 1 Deterioration	BP 102/65	Lower perfusion than baseline	Important	Shows illness progression	Miss hemodynamic decline	Trend BP	Ongoing	A	Relative hypotension matters		No
Phase 1 Deterioration	RR 20	Respiratory rate stable but oxygenation poor	Relevant	Must interpret with SpO2	Miss respiratory compromise	Assess overall respiratory status	Ongoing	B	Normal RR does not cancel hypoxia		Yes
Phase 1 Deterioration	SpO2 89 percent on room air	Hypoxia	Immediate	Below target and already symptomatic	Organ dysfunction and worsening mentation	Apply oxygen promptly	Reassess saturation and lungs	A R	Do not ignore borderline hypoxia in sick postop patient	✓	Yes
Phase 1 Deterioration	Temp 36.9	No fever currently	Relevant	Does not rule out infection	False reassurance	Continue full assessment	N/A	B	Absence of fever does not exclude serious illness		Yes
Phase 1 Deterioration	Pain 2 out of 10 at incision	Low pain not main issue	Relevant	Do not anchor on pain	Minimal	Monitor but prioritize instability	N/A	B	Low pain does not mean stable		Yes
Phase 1 Deterioration	PVCs 30 per minute	Abnormal cardiac ectopy from severe hyperkalemia	Immediate	Major cardiac red flag	Progression to ventricular arrhythmia or arrest	Obtain EKG treat hyperkalemia immediately	Reassess telemetry rhythm	A + R	PVCs plus high potassium is an emergency	✓	Yes
Phase 1 Deterioration	Confused situation time and place	Significant encephalopathy	Immediate	May reflect hypoxia uremia or systemic illness	Miss worsening organ dysfunction	Perform neuro assessment and protect safety	Reassess mentation	A R	Mentation is a major outcome measure	✓	Yes
Phase 1 Deterioration	Incision red swollen purulent open to air	Active infected wound	Important	Source of illness still present	Sepsis progression	Assess and report wound	Ongoing	B	Do not lose the infection source while treating potassium		No
Phase 1 Deterioration	30 mL dark amber urine	Oliguria with concentrated urine	Immediate	Clear sign of poor renal function	Miss AKI severity	Monitor output closely	Reassess after furosemide	A R	Output is a key endpoint of treatment	✓	Yes

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Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	SBAR	Teaching/Debrief Focus	High Risk	Common Missed Items
Phase 1 Deterioration	Yellow green emesis	Ongoing vomiting	Important	Symptom burden aspiration risk and instability	Delayed antiemetic support or aspiration	Administer ondansetron	Reassess nausea	B	Supportive care matters too		No
Phase 1 Deterioration	2 plus bilateral lower leg edema	Fluid overload clue	Important	Supports renal dysfunction and possible pulmonary congestion	Miss volume status problem	Assess edema lungs and response to diuresis	Reassess after furosemide	B	Edema plus crackles plus low urine output fit together	✓	Yes
Phase 1 Expected Actions	Review lab values	Core preparation step	Immediate	Labs reveal the actual emergency	Poor hypothesis or delayed treatment	Review and interpret labs before provider call	N/A	R	Available data should drive thinking	✓	Yes
Phase 1 Expected Actions	Obtain vital signs	Required reassessment step	Immediate	Confirms severity and trends	Miss deterioration	Obtain and trend full vitals	Repeat after interventions	A	Assessment before escalation	✓	Yes
Phase 1 Expected Actions	Perform assessment	Full bedside assessment required	Immediate	Links labs to patient presentation	Delay in appropriate care	Assess neuro cardiac respiratory GI GU wound and volume status	Repeat after interventions	A	Assessment drives prioritization	✓	Yes
Phase 1 Expected Actions	Hold Enalapril	Prevents worsening potassium and renal stress	Immediate	Important medication safety step	Worsening hyperkalemia or AKI	Do not administer and communicate reason	Reassess BP and plan	B R	Students should know when to hold meds	✓	Yes
Phase 1 Expected Actions	Administer Ondansetron	Appropriate supportive care for active vomiting	Important	Improves comfort and safety	Aspiration or inability to proceed with care	Give antiemetic	Reassess nausea	B	Supportive care can happen while preparing urgent care		No
Phase 1 Expected Actions	Form hypothesis and SBAR before calling MD	Organized escalation	Important	Improves provider communication and recommendations	Weak communication and delay	Prepare focused SBAR	N/A	S A R	SBAR should include K creatinine urine output PVCs and confusion	✓	Yes
Phase 1 Student Cue	Patient actively vomiting	Confirms symptom severity	Important	Adds urgency and aspiration risk	Delayed symptom treatment	Protect airway and medicate	Reassess nausea	B	Patient cues should be taken seriously		No
Phase 1 Student Cue	Patient thinks it is Christmas and is anxious	Clear acute confusion	Immediate	Major neuro change and safety concern	Missed encephalopathy	Reorient and assess cause	Reassess mentation	A R	Confusion is a sign not just a behavior problem	✓	Yes

Post-Operative Complications DeShawn Johnson

Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	SBAR	Teaching/Debrief Focus	High Risk	Common Missed Items
Phase 1 Student Cue	Patient knows only name DOB and surgery	Severely limited orientation	Immediate	Shows worsening mentation	Miss progression	Trend neuro status	Reassess after treatment	A R	Acute neuro change must be tracked		Yes
Phase 1 Orders	Stat 12 lead EKG	Needed to define cardiac effects of hyperkalemia	Immediate	Abnormal potassium plus PVCs requires EKG	Missed unstable rhythm	Obtain EKG promptly	Interpret and reassess	A + R	EKG is required not optional here	✓	Yes
Phase 1 Orders	Discontinue vancomycin	Stops possible nephrotoxic injury	Immediate	Protects kidneys from further harm	Continued renal injury	Stop infusion	Reassess med status	A R	Remove the offending contributor	✓	Yes
Phase 1 Orders	CMP 2 hours after furosemide	Follow-up effectiveness lab	Important	Confirms response	Miss persistent hyperkalemia	Obtain repeat lab	Review results	R	Reassessment includes lab follow-up	✓	Yes
Phase 1 Orders	Calcium gluconate now	Cardiac membrane stabilization	Immediate	First-line cardiac protection	Progression to lethal arrhythmia	Administer now	Reassess rhythm	A R	Calcium protects the heart first	✓	Yes
Phase 1 Orders	D50 now	Prevents hypoglycemia with insulin	Immediate	Needed with insulin therapy	Hypoglycemia	Administer with insulin	Reassess glucose	A R	Insulin and dextrose are paired	✓	Yes
Phase 1 Orders	Regular insulin IV now	Shifts potassium intracellularly	Immediate	Core treatment for severe hyperkalemia	Persistent hyperkalemia	Administer promptly	Reassess glucose rhythm labs	A R	Know mechanism and monitor for hypoglycemia	✓	Yes
Phase 1 Orders	Furosemide IV now	Promotes potassium excretion and improves volume status	Immediate	Important if patient can make urine	Continued hyperkalemia and overload	Administer promptly	Reassess urine output lungs and BP	A R	Look for response in urine output and lungs	✓	Yes
Phase 1 Orders	Make client NPO	Protects from aspiration and prepares for worsening condition	Important	Appropriate with vomiting and instability	Aspiration or unsafe PO intake	Enforce NPO	Reassess orders	B	Orders should evolve with patient status		No
Expected Reassessment	Increased urine output	Key sign treatment is working	Immediate	Shows renal response and diuretic effect	Miss failed treatment	Measure output carefully	Trend I&O	A R	Urine output is a core endpoint	✓	Yes
Expected Reassessment	Improved oxygenation	Key sign supportive care and fluid management are helping	Important	Shows respiratory status response	Miss persistent hypoxia	Reassess SpO2 and need for oxygen	Trend saturation	A R	Oxygenation is an outcome measure		No
Expected Reassessment	Improved lung sounds	May show reduced congestion	Important	Helps assess fluid balance	Miss continued overload	Reassess crackles and breathing	Trend respiratory assessment	B	Listen again after treatment		No

Post-Operative Complications DeShawn Johnson

Section	Finding	Clinical Significance	Priority	Priority Rationale	Risk if Missed	Expected Nursing Action	Reassessment	SBAR	Teaching/Debrief Focus	High Risk	Common Missed Items
Expected Reassessment	Improved mentation	Major marker of perfusion and metabolic improvement	Immediate	Shows neurologic recovery	Miss ongoing encephalopathy	Reassess orientation and confusion	Trend LOC	A R	Mentation should improve if treatment works	✓	Yes
Scenario-Wide Teaching Point	Hyperkalemia plus PVCs is a cardiac emergency	Core learning point	Immediate	Students must recognize lethal risk	Cardiac arrest	Treat immediately and escalate	N/A	A	Do not wait for the patient to crash	✓	Yes
Scenario-Wide Teaching Point	AKI is shown by creatinine trend and oliguria	Core renal learning point	Immediate	This is not subtle renal dysfunction	Delayed recognition and harm	Use trend data and output together	N/A	B	Creatinine and urine output tell the story	✓	Yes
Scenario-Wide Teaching Point	Reassessment after treatment is required	Core nursing principle	Immediate	Must know whether treatment worked	Miss failed response	Repeat vitals rhythm mentation oxygenation and I&O	Ongoing	A R	Even no change is important if reassessed	✓	Yes
Scenario-Wide Teaching Point	Do not focus only on infection	Common reasoning error	Important	Students may miss the bigger emergent problem	Wrong priorities	Treat immediate threats first while addressing source	N/A	B	Source control matters but cardiac risk comes first	✓	Yes

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Section	Debrief Question	Instructor Prompt
Reaction / Defusing	How did you feel throughout the simulation experience?	Encourage students to identify their initial reaction before moving into analysis.
Reaction / Defusing	Give a brief summary of this patient and what happened in the simulation.	Guide students to summarize the transition from post-op infection to AKI and hyperkalemia emergency.
Reaction / Defusing	What were the main problems that you identified?	Prompt students to identify hyperkalemia AKI low urine output abnormal EKG and confusion as the primary problems.
Analysis / Discovery	Discuss the knowledge guiding your thinking surrounding these main problems.	Encourage students to connect lab abnormalities to cardiac renal and neurologic findings.
Analysis / Discovery	What were the key assessments and interventions for this patient?	Guide students toward lab review telemetry EKG oxygenation urine output calcium insulin dextrose furosemide and stopping vancomycin.
Analysis / Discovery	Discuss how you identified these key assessments and interventions.	Prompt students to explain why potassium creatinine BUN PVCs and urine output made the situation urgent.
Analysis / Discovery	Discuss the information resources you used to assess this patient. How did this guide your care planning?	Encourage students to include shift report labs telemetry medication review and bedside assessment.
Analysis / Discovery	Discuss the clinical manifestations evidenced during your assessment. How would you explain these manifestations?	Guide students toward confusion vomiting oliguria PVCs edema and hypoxia as manifestations of renal dysfunction hyperkalemia and post-op complications.
Analysis / Discovery	Explain the nursing management considerations for this patient. Discuss the knowledge guiding your thinking.	Prompt students to prioritize cardiac stabilization renal support oxygenation medication safety and reassessment.
Analysis / Discovery	What information and information management tools did you use to monitor this patient's outcomes? Explain your thinking.	Encourage students to identify telemetry EKG vital signs urine output pulse oximetry and repeat labs.

Section	Debrief Question	Instructor Prompt
Analysis / Discovery	How did you communicate with the patient?	Guide students to reflect on calm reorientation and clear communication with a confused patient.
Analysis / Discovery	What specific issues would you want to take into consideration to provide for this patient's unique care needs?	Prompt students to consider confusion fall risk post-op infection nicotine dependence and medication safety.
Analysis / Discovery	Discuss the safety issues you considered when implementing care for this patient.	Encourage focus on dysrhythmia risk aspiration hypoxia falls medication holding and monitoring during IV pushes.
Analysis / Discovery	What measures did you implement to ensure safe patient care?	Guide students toward holding Enalapril applying oxygen stopping vancomycin using telemetry and reassessing after treatment.
Analysis / Discovery	What other members of the care team should you consider important to achieving good care outcomes?	Prompt students to include provider pharmacy lab telemetry support and possibly nephrology depending on progression.
Analysis / Discovery	How would you assess the quality of care provided?	Encourage evaluation of timeliness prioritization communication and reassessment.
Analysis / Discovery	What could you do to improve the quality of care for this patient?	Prompt students to focus on earlier recognition stronger lab interpretation and better linking of labs to cardiac findings.
Summary / Application	If you were able to do this again how would you handle the situation differently?	Encourage students to identify one or two specific behavior changes.
Summary / Application	What did you learn from this experience?	Guide students toward a clear takeaway about hyperkalemia AKI and reassessment.
Summary / Application	How will you apply what you learned today to your clinical practice?	Prompt students to connect abnormal lab interpretation and telemetry changes to future bedside practice.
Summary / Application	Is there anything else you would like to discuss?	Use this as an invitation for unresolved concerns or reflection.

Postoperative Complications

Students are expected to identify postoperative complications such as hyperkalemia and acute kidney injury, intervene appropriately, and reassess the patient's response.

Critical Cues

- ☐ Elevated potassium
- ☐ Low urine output
- ☐ Confusion
- ☐ Abnormal ECG
- ☐ Wound infection

Expected Priorities

- ☐ Hyperkalemia
- ☐ Kidney function
- ☐ Oxygenation

Expected Interventions

- ☐ Hold contraindicated medication
- ☐ Treat hyperkalemia
- ☐ Notify provider
- ☐ Monitor cardiac rhythm

Expected Reassessment

- ☐ Urine output
- ☐ Mentation
- ☐ Cardiac rhythm

Common Student Errors

- Missing hyperkalemia
- Treating nausea first
- Forgetting telemetry
- Not reassessing urine output

CJMM-ALIGNED SIMULATION DEBRIEFING QUICK REFERENCE GUIDE

This guide provides flexible reflection prompts aligned with the NCSBN Clinical Judgment Measurement Model (CJMM). Select, adapt, and sequence questions based on simulation objectives, student performance, and learner needs. Use the prompts as a cognitive aid to support discussion and reflection, not as a required script or checklist.

1. Recognize Cues
- What patient findings or changes in condition stood out to you?
- Which assessment findings were most important?
- What changes suggested that the patient's condition was improving or worsening?
- Were there any important cues that were initially missed or overlooked?

2. Analyze Cues
- What did the assessment findings suggest was happening with the patient?
- How were the important cues related to one another?
- How did the patient's pathophysiology help explain the findings?
- What additional information would have helped you better understand the patient's condition?

3. Prioritize Hypotheses
- What did you believe was the patient's highest-priority problem?
- Which patient problem or concern required attention first, and why?
- What was the greatest immediate risk to the patient?
- How did you decide which problem was most urgent?

4. Generate Solutions
- What nursing interventions did you consider for the patient's priority problem?
- What outcome did you expect from those interventions?
- What other interventions or approaches could have been considered?
- What additional resources, information, or team members could have helped guide the plan of care?

5. Take Action
- What action did you take first, and why?
- How did the patient's condition influence the order of your interventions?
- Were there any actions that should have been taken earlier or differently?
- How did communication and teamwork affect the care provided?

6. Evaluate Outcomes
- How did you determine whether the patient responded to the interventions?
- What findings needed to be reassessed after the interventions?
- Did the patient's response require a change in the plan of care?
- What would you continue to monitor or reassess if care of this patient continued?

7. Reflection and Future Clinical Application
- What went well, and what challenged you the most?
- What would you do differently if you encountered a similar situation again?
- How will you apply what you learned from this experience to future patient care?